

PREVENTION MODULE

What to remember

Online Module Quick Reference

Each lesson of the online module is distilled to its four or five essential ideas. This document does not replace the manual, the video capsules or in-class practice. It gives you a reference point before the in-person session and a quick refresher after the course.

Knowledge online, skills in class. Arrive at the in-person session with these points mastered.

MODULE CONTENTS: SIX LESSONS

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00

Introduction to the chapter

Five lessons to anticipate risk before having to treat it

Why this chapter: in the backcountry, help is far away. The best intervention is the one you never had to make.

- 1 Prevention beats treatment.** Most accidents in remote settings are preventable through good preparation, adequate supervision and continuous reading of the risk.
- 2 The wilderness first aider does more than an urban first aider.** Prolonged delays, limited resources, independent decisions: your role covers prevention, patient care and evacuation management.
- 3 The module follows a logical progression.** Role of the first aider, leadership and planning, legal framework, protection against communicable diseases, then how the body works and how your interventions support it.
- 4 Knowledge online, skills in class.** What you learn here will be applied in person: glove removal, scene survey, group management, scenarios.

LEARNER'S MANUAL

Section 2, Wilderness First Aid · Online encyclopedia

FIELD MANUAL

Pages 11 to 20, Prevention section

IN CLASS

Icebreaker: leadership, communication, scene safety, glove removal

01

First aid

What changes when help is far away

The first aider's objective: to provide the best possible care, as quickly as possible, without creating new casualties.

	Urban setting	Wilderness
Access to care	Minutes	Hours to days
Resources	Available, specialized	Limited, improvised
Communications	Easy, 9-1-1	Limited, local numbers
Environment	Controlled	Hostile, changing

Figure 1. How delayed access to care changes your role.

- 1 **Four basic principles, everywhere and always.** Ensure the safety of everyone present, prevent further injury, perform life-saving interventions, and hand over responsibility for care to prehospital services as soon as possible.
- 2 **Urban setting and backcountry: two realities.** See the figure. Delay transforms the role: you are not just stabilizing while you wait, you are taking charge of care.
- 3 **Know the local emergency network.** Ground emergency medical services (first responders, paramedics), air EMS, fire, police, search and rescue. 9-1-1 does not work everywhere: the numbers that actually work must be in the emergency plan.
- 4 **First aider or health care professional.** Level of training and context dictate the degree of care to provide. The first aider must recognize their own limits when faced with a critically injured patient.
- 5 **The qualities expected of a first aider.** Stay calm, organized and diligent, act professionally, keep your skills current, and be a role model through a healthy and safe lifestyle.

02

Leadership

Your responsibilities in prevention and risk management

The leader is anyone responsible for a group in a remote setting: expedition leader, guide, camp director, worksite supervisor.



Figure 2 — La prévention se joue avant le départ, dans cet ordre.

- 1 **A competent leader, a clear role.** Physical fitness, experience and training suited to the activity. When several leaders supervise a group, roles are defined in advance. The leader-to-participant ratio follows the guidelines for the activity.
- 2 **Match the group to the activity.** A mismatch between members' abilities and the demands of the trip is a frequent cause of accidents. Pre-trip meetings and medical forms allow problems to be anticipated, rarely to exclude anyone.
- 3 **A risk management plan.** Assess the severity of risks, then prevent, control, reduce or eliminate them. Inform clients and workers of inherent risks, ideally through a safety briefing and a written document.
- 4 **A written emergency plan.** Leader roles, predetermined options, means of communication, exit points and evacuation routes marked on a map, emergency contacts in a waterproof container, scheduled review date.
- 5 **Trip plan, region and local hazards.** Leave the trip plan with a designated contact who will alert authorities if you are overdue, and notify them on your return. Know the access points, exits, local services and their limits. Check the diseases and hazards of the region and plan vaccination early enough for it to be effective.

03

Laws and responsibilities

Know your duties so you can act with confidence

Two legal systems in Canada (civil law in Quebec, common law elsewhere), one shared premise: everyone must behave reasonably.

- 1 **Duty and standard of care.** The leader has a legal obligation to look after the group. The standard of care is the one set by peers, industry standards and common sense: you provide only the care that matches your training. Negligence is a breach of that standard that causes foreseeable harm.
- 2 **Protection of the good-faith rescuer.** In Quebec, a person who comes to another's aid is exempt from liability, except in cases of intentional or gross fault (Civil Code, art. 1471). The Quebec Charter requires coming to the aid of anyone whose life is in peril, unless there is a risk to oneself or others (s. 2). An employee first aider acting at the employer's request is protected (Canada Labour Code, s. 126(3)).
- 3 **Consent before care.** Informed consent: a conscious, capable person is informed, then agrees. Implied consent: an unconscious person is presumed to consent. Minors: parents or guardians, or the responsible adult (in loco parentis). Incapacity (intoxication, altered mental status) changes the assessment of capacity.
- 4 **The right to refuse.** Any person of legal age who is capable may refuse treatment. Document the refusal, explain the risks, remain available and reassess.
- 5 **Confidentiality, abandonment, documentation.** Disclose information only to those taking over care. Once care has begun, do not stop without handing over to someone of equal or higher training. The incident report completed on site is a legal document: keep it.

04

Preventing disease transmission

The risks present, and how to protect yourself

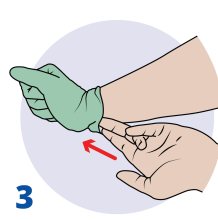
A communicable disease spreads from one individual to another. Protecting yourself also protects the patient and the rest of the group.



1. Pincer l'extérieur du premier gant au poignet.



2. Le retirer en le retournant, sans toucher la peau.



3. Glisser deux doigts nus sous le second gant.



4. Envelopper le premier gant dans le second, puis jeter.

Figure 3 — Retrait des gants souillés, pratiqué en classe.

- 1 Four modes of transmission.** Direct contact (blood, body fluids), indirect contact (objects, soiled dressings), airborne transmission (droplets, aerosols) and vector-borne transmission (ticks, mosquitoes, animals).
- 2 Universal precautions.** Treat all blood and all body fluids as potentially infectious, in every patient, at all times.
- 3 Personal protective equipment.** Gloves, pocket mask for ventilation, goggles or face shield, protective clothing. Surgical mask for droplets, N95 for aerosols.
- 4 Glove removal and hand washing.** Remove gloves without touching their outer surface (technique covered in class), then wash your hands: it is the single most effective measure against transmission. Clean reusable equipment and dispose of soiled supplies safely.
- 5 Exposure and immunization.** If exposed to a body fluid: wash thoroughly, document, seek medical advice promptly. Keep your immunizations current, especially tetanus, and consider rabies depending on the exposure and the region.

LEARNER'S MANUAL

Section 2.3, Preventing Disease
Transmission · Online encyclopedia

FIELD MANUAL

Pages 16 and 17,
protection and glove
removal

IN CLASS

Workshop: removing soiled gloves without
spreading fluid

05

Structures of the body

Understanding the natural mechanisms your interventions support

Homeostasis is the continuous maintenance of a stable internal environment. Every emergency is a disruption of that balance.

- 1 From cell to system.** Cells, tissues, organs, systems. The cell is the basic unit: it converts nutrients into energy and heat and requires a constant supply of oxygen and glucose.
- 2 The systems are interdependent.** The respiratory system brings in oxygen, the cardiovascular system delivers it to the cells, the nervous system detects and adjusts, and the kidneys and digestive system regulate fluids and nutrients.
- 3 Seven essential elements.** Water, oxygen, glucose, electrolytes, blood pressure, temperature, acid-base balance (pH). Each must stay within a narrow range.
- 4 Oxygen first.** Heart and brain cells are the most sensitive to lack of oxygen: irreversible damage after 4 to 6 minutes. Airway and breathing always take precedence over local injuries.
- 5 Read every emergency through these seven elements.** Hemorrhage, hypothermia, dehydration, hypoglycemia, shock: each time, one or more elements move out of their range. Your actions aim to bring them back: warmth, hydration, sugar, oxygen, perfusion.